

6 November 2024

Joint DMMAG and ICB Letter to healthcare professionals re: tirzepatide and GLP-1 RA prescribing in Birmingham and Solihull

Summary of key points:

- Tirzepatide (Mounjaro®) is currently only commissioned for use in patients with Type 2 diabetes to improve glycaemic control. **It should NOT be prescribed for weight management until further guidance is available.** Any requests to prescribe it for weight management should be declined.
- Tirzepatide can be considered for diabetes in line with [NICE TA 294](#) if triple therapy with metformin and 2 other oral drugs is ineffective, not tolerated or contraindicated. Prescribers can switch one oral drug for a GLP-1 RA or tirzepatide in patients with raised BMI (> 35, or lower in BAME background) or if insulin is not recommended
- Tirzepatide and GLP-1 RAs should not be prescribed for people with Type 1 diabetes
- National shortages of some GLP-1 RAs are ongoing, however oral semaglutide (Rybelsus®) is available and supply can support an uplift in demand. Where Rybelsus is unsuitable, tirzepatide can be considered
- Off-label use of GLP1-RAs for weight management and the use for other indications (e.g. for CVD prevention) is non-formulary and should be avoided
- Local availability and access to GLP-1 RAs licensed for weight management e.g. liraglutide (Saxenda®) and injectable semaglutide (Wegovy®) - is extremely limited or unavailable
- Prescribers are reminded to advise patients about the side effects of GLP-1 RAs and to report misuse

Tirzepatide: local commissioning status

The dual GLP-RA/GIP agent, tirzepatide (Mounjaro®), has received significant media attention since its launch earlier this year, creating unprecedented demand for prescribing. We already have nearly £1m prescribing spend in primary care across NHS Birmingham and Solihull in the 2024/25 financial year to date, and further growth is forecast. This is in considerable excess of expectations based on NICE criteria.

Whilst licensed for weight loss and diabetes, tirzepatide is only commissioned locally for the management of type 2 diabetes, in line with NICE Technology Appraisal ([TA924](#)). The Diabetes Medicines Management Advisory Group (DMMAG) in BSol clarified the positioning of tirzepatide in its recently updated [type 2 diabetes guideline](#) published in June 2024; pages 10 and 11 contain a useful 2-page summary that outlines the place in therapy for all antidiabetic agents, including GLP1-RAs and tirzepatide.

Tirzepatide should be considered **only if triple therapy with metformin and 2 other oral drugs is not effective, not tolerated or contraindicated**. Prescribers can consider switching one oral drug for a GLP-1 RA or GIP/GLP-1 RA in those patients whose BMI is 35 kg/m² or more (lower in Black, Asian and other minority ethnic groups) or if insulin is not recommended due to occupation or co-morbidities.

We are already seeing patients being prescribed these agents outside this criteria, for example as monotherapy, or in dual therapy regimes with metformin or insulin alone, and for patients with otherwise good glycaemic control. Many cases appear to be based on the fact that a diabetic patient is overweight as the primary decision factor. We would remind prescribers that the use of GLP1-RAs and tirzepatide in patients with diabetes should be for glycaemic benefit only; weight loss is an added benefit. The optimal use of other oral agents first, such as SGLT2i's, will also have additional weight loss benefits.

DMMAG recommends that tirzepatide doses up to 10mg weekly will be sufficient for glycaemic control in most patients. In fact, studies show about 80% achieve target HbA1c on 5mg weekly. Prescribers in primary care should seek specialist advice before increasing doses above 10mg. Please note that GLP-1 RAs and tirzepatide should not be prescribed for people with Type 1 diabetes.

Shortages of GLP-1 RAs

We recognise that the national shortages of GLP-1 RAs has been a contributing factor in the rise of tirzepatide prescribing. Oral semaglutide (Ryblesus®) is a viable GLP-1 RA option with sufficient supply available to support an uplift in demand, this can be considered as a first line option in this scenario due to its lower cost. At the time of writing, dulaglutide (Trulicity®) and semaglutide (Ozempic®) supplies remain limited or intermittent, whilst exenatide immediate release (Byetta®) and liraglutide (Victoza®) have now been discontinued.

GLP-1 RAs for weight loss and other indications

There is also considerable interest in the use of GLP-1 RAs and tirzepatide for weight management and NICE guidance on tirzepatide is expected imminently. We are already seeing increased prescribing for weight management indications, and a

small number of requests for injectable semaglutide (Wegovy®) that appear to be based on reduction of cardiovascular risk.

As per local and national recommendations, when prescribing GLP-1 RAs for weight loss, they should be licensed for this purpose. Off-label prescribing of GLP-1 RAs solely for weight management in patients who do not have diabetes is not recommended. Use of GLP-1 RAs for the prevention of cardiovascular disease or other indications is also non-formulary.

NICE guidance for tirzepatide in weight management is expected later in the year, and it is highly likely that an NHS England funding variation request will be in place which will recommend a phased roll out taking place over a number of years, concentrating on those with the highest clinical need first. Once published, we will have to consider what local implementation will look like. **Until this is clear, prescribers are requested not to recommend or initiate tirzepatide for weight management.**

Unfortunately the availability of other GLP-1 RAs licensed for weight management remains limited: the shortage of liraglutide (Saxenda®) is ongoing, and neither of the licensed GLP-1 RAs (Saxenda® nor injectable semaglutide (Wegovy®)) should be prescribed in primary care for weight management indications as per NICE TAs; the referral criteria for local weight management services has also changed from 4 November (see ICB letter 25 October 2024 in the recent ICB Primary Care Bulletin).

Risk of harm

Despite widespread use of these agents, there remains some risk of harm. The MHRA has reminded healthcare professionals to advise patients of the side effects of GLP-1 RAs and to report misuse. Gastrointestinal side effects are very common and may affect more than 1 in 10 patients, if severe this can lead to dehydration. Hypoglycaemia can occur in non-diabetic patients using some GLP-1 RAs for weight management. Other serious but less common side-effects of GLP-1 RAs include acute gallstone disease, pancreatitis, and serious allergic reactions. See MHRA [Drug Safety Update](#), October 2024, for further guidance.

We hope this letter serves to clarify the local positioning of GLP-1 RA agents and tirzepatide, both for diabetes and for weight management. We seek the support of our local healthcare professionals to help us maintain appropriate and equitable access to these medicines, constrained within a financially-challenged NHS environment.

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